Effective Date: 08/20/2025
Last Approval/Version: 10/2026
Next Review Due By: 08/2027
Policy Number: C29655-A

Adalimumab and Biosimilars, IL Medicaid Only

PRODUCTS AFFECTED

Abrilada (adalimumab-afzb), adalimumab-aacf, adalimumab-aaty, adalimumab-adaz, adalimumab-adbm, adalimumab-bwwd, adalimumab-fkjp, adalimumab-ryvk, Amjevita (adalimumab-atto), Cyltezo (adalimumab-adbm), Hadlima (adalimumab-bwwd), Hulio (adalimumab-fkjp), Humira (adalimumab), Hyrimoz (adalimumab-adaz), Idacio (adalimumab-aacf), Simlandi (adalimumab-ryvk), Yuflyma (adalimumab-aaty), Yusimry (adalimumab-aqvh)

COVERAGE POLICY

Coverage for services, procedures, medical devices and drugs are dependent upon benefit eligibility as outlined in the member's specific benefit plan. This Coverage Guideline must be read in its entirety to determine coverage eligibility, if any. This Coverage Guideline provides information related to coverage determinations only and does not imply that a service or treatment is clinically appropriate or inappropriate. The provider and the member are responsible for all decisions regarding the appropriateness of care. Providers should provide Molina Healthcare complete medical rationale when requesting any exceptions to these guidelines.

Documentation Requirements:
Molina Healthcare reserves the right to require that additional documentation be made available as part of its coverage determination; quality improvement; and fraud; waste and abuse prevention processes. Documentation required may include, but is not limited to, patient records, test results and credentials of the provider ordering or performing a drug or service. Molina Healthcare may deny reimbursement or take additional appropriate action if the documentation provided does not support the initial determination that the drugs or services were medically necessary, not investigational or experimental, and otherwise within the scope of benefits afforded to the member, and/or the documentation demonstrates a pattern of billing or other practice that is inappropriate or excessive.

DIAGNOSIS:
Rheumatoid arthritis (RA), Juvenile idiopathic arthritis, Psoriatic arthritis, Plaque psoriasis, Crohn's disease, Ulcerative colitis, Ankylosing spondylitis, Hidradenitis suppurativa, Non-infectious uveitis, Pyoderma gangrenosum

REQUIRED MEDICAL INFORMATION:
This clinical policy is consistent with standards of medical practice current at the time that this clinical policy was approved. This clinical policy will be reviewed along with state and federal requirements, the benefit being administered, and formulary preferencing. If a drug within this policy receives an updated FDA label within the last 180 days, medical necessity for the member will be reviewed using the updated FDA label information. Coverage will be determined on a case-by-case basis until the criteria can be updated through Molina Healthcare, Inc. clinical governance. Additional information may be required on a case-by-case basis to allow for adequate review.

A. FOR ALL INDICATIONS:
   1. Prescriber attests member does not have an active or latent untreated infection (e.g., Hepatitis B, tuberculosis, etc.), including clinically important localized infections, according to the FDA label

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AND

2. Member is not on concurrent treatment or will not be used in combination with other TNF-inhibitor, biologic response modifier or other biologic DMARDs, Janus kinase Inhibitors, or Phosphodiesterase 4 inhibitor (i.e., apremilast, tofacitinib, baricitinib) as verified by prescriber attestation, member medication fill history, or submitted documentation
   AND
3. If request is for a NON-FORMULARY/NON-PREFERRED reference product with a biosimilar available for initial or continuation of therapy requests: Documentation of a trial and failure, serious side effects or contraindication to a preferred biosimilar product is required (unless there is data demonstrating clinical superiority of reference drugs over the FDA approved biosimilar drugs).
   [DOCUMENTATION REQUIRED: Document when the preferred biologic product or biosimilar was tried and the length of the trial period. Provide specific clinical documentation of therapeutic failure on the preferred biologic product or biosimilar whenever possible. Describe the medical problem caused by the preferred referenced biologic. Vague and non-descriptive symptoms are not adequate rationale (e.g., stomachache).]
   PREFERRED AGENTS: Hadlima (adalimumab-bwwd), Simlandi (adalimumab-ryvk) and adalimumab- adbm. [Note: Some package sizes may be non-preferred. Please consult IL Medicaid PDL for more details]

B. MODERATE TO SEVERE RHEUMATOID ARTHRITIS:
   1. Documentation of moderate to severe rheumatoid arthritis diagnosis
      AND
   2. Documentation of prescriber baseline disease activity evaluation and goals for treatment to be used to evaluate efficacy of therapy at renewal [DOCUMENTATION REQUIRED]
      AND
   3. (a) Member is currently receiving maximally tolerated dose of methotrexate and is not at goal disease activity
      OR
      (b) Member has an FDA labeled contraindication or serious side effects to methotrexate, as determined by the prescribing physician AND Member has tried one additional disease-modifying antirheumatic drug (DMARD) (brand or generic; oral or injectable) for at least 3 months NOTE: An exception to the requirement for a trial of one conventional synthetic DMARD can be made if the member has already had a 3-month trial of at least one biologic. These members who have already tried a biologic for RA are not required to “step back” and try a conventional synthetic DMARD.

C. JUVENILE IDIOPATHIC ARTHRITIS:
   1. Documented diagnosis of systemic juvenile idiopathic arthritis (SJIA) or non-systemic juvenile idiopathic arthritis (polyarthritis, oligoarthritis, enthesitis, dactylitis, TMJ arthritis) in a member that had symptom onset prior to age 16
      AND
   2. Documentation of prescriber baseline disease activity evaluation and goals for treatment to be used to evaluate efficacy of therapy at renewal [DOCUMENTATION REQUIRED]
      AND
   3. FOR NON-SYSTEMIC JUVENILE IDIOPATHIC ARTHRITIS: Documentation of treatment failure, serious side effects or clinical contraindication to an adequate trial (generally ≥12 weeks) of one or more of the following: Methotrexate, hydroxychloroquine, sulfasalazine, leflunomide

D. PSORIATIC ARTHRITIS:
   1. Documentation of active psoriatic arthritis
      AND
   2. Documentation of prescriber baseline disease activity evaluation and goals for treatment to be used to evaluate efficacy of therapy at renewal [DOCUMENTATION REQUIRED]

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AND

3. (a) Documented treatment failure, serious side effects or clinical contraindication to a minimum 3-month trial of ONE of the following: leflunomide, methotrexate, sulfasalazine, cyclosporine
   OR
   (b) Documentation member has severe psoriatic arthritis [erosive disease, elevated markers of inflammation, long term damage that interferes with function, highly active disease that causes a major impairment in quality of life, active PsA at many sites including dactylitis, enthesitis, function-limiting PsA at a few sites or rapidly progressive disease]
   OR
   (c) Documentation member has severe psoriasis [PASI ≥ 12, BSA of >5-10%, significant involvement in specific areas (e.g., face, hands or feet, nails, intertriginous areas, scalp), impairment of physical or mental functioning with lower amount of surface area of skin involved]

E. CHRONIC PLAQUE PSORIASIS

1. Documented diagnosis of moderate to severe psoriasis (BSA ≥ 3%) OR < 3% body surface area with plaque psoriasis that involves sensitive areas of the body or areas that would significantly impact daily function (e.g., face, neck, hands, feet, genitals)
   AND
2. (a) Documentation of treatment failure, serious side effects, or clinical contraindication to TWO systemic therapies for ≥3 months (e.g., methotrexate, cyclosporine, azathioprine, hydroxyurea, leflunomide, mycophenolate mofetil, or tacrolimus)
   OR
   (b) Documentation of treatment failure to Phototherapy for ≥3 months with either psoralens with ultraviolet A (PUVA) or ultraviolet B (UVB) radiation. Provider to submit documentation of duration of treatment, dates of treatment, and number of sessions
   OR
   (c) Documentation of contraindications to systemic therapy and phototherapy
   NOTE: Contraindications to phototherapy include type 1 or type 2 skin, history of photosensitivity, treatment of facial lesions, presence of premalignant lesions, history of melanoma or squamous cell carcinoma, or physical inability to stand for the required exposure time
   AND
3. Documentation of prescriber baseline disease activity evaluation and goals for treatment to be used to evaluate efficacy of therapy at renewal [DOCUMENTATION REQUIRED]

F. CROHN'S DISEASE

1. Documentation of a diagnosis of Crohn’s Disease
   AND
2. Member has one or more high risk feature:
   i. Diagnosis at a younger age (<30 years old)
   ii. History of active or recent tobacco use
   iii. Elevated C-reactive protein and/or fecal calprotectin levels
   iv. Deep ulcers on colonoscopy
   v. Long segments of small and/or large bowel involvement
   vi. Perianal disease
   vii. Extra-intestinal manifestations
   viii. History of bowel resections
   AND
3. (a) Documentation of treatment failure, serious side effects or clinical contraindication to an adequate trial (> 3 months) of ONE immunomodulator (e.g., azathioprine, 6-mercaptopurine, methotrexate) up to maximally indicated doses
   OR
   (b) Prescriber provides documented medical justification that supports the inability to use immunomodulators:

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i. Inability to induce short-term symptomatic remission with a 3-month trial of systemic glucocorticoids
ii. High-risk factors for intestinal complications may include: Initial extensive ileal, ileocolonic, or proximal GI involvement, initial extensive perianal/severe rectal disease, fistulizing disease (e.g., perianal, enterocutaneous, and rectovaginal fistulas), deep ulcerations, penetrating, stricturing or stenosis disease and/or phenotype, intestinal obstruction, or abscess
iii. High risk factors for postoperative recurrence may include: Less than 10 years duration between time of diagnosis and surgery, disease location in the ileum and colon, perianal fistula, prior history of surgical resection, use of corticosteroids prior to surgery

AND

4. Documentation of prescriber baseline disease activity evaluation and goals for treatment to be used to evaluate efficacy of therapy at renewal [DOCUMENTATION REQUIRED]

G. ULCERATIVE COLITIS
1. Documentation of ulcerative colitis diagnosis with evidence of moderate to severe disease activity
AND
2. (a) Documentation of treatment failure, serious side effects or clinical contraindication to a 2-month trial of one systemic agent (e.g., 6-mercaptopurine, azathioprine, cyclosporine, tacrolimus, or a corticosteroid such as prednisone, methylprednisolone) for ulcerative colitis or will continue to take concurrently.
NOTE: A previous trial of a biologic (e.g., an adalimumab product [e.g., Humira], Simponi SC [golimumab SC injection], or Entyvio [vedolizumab IV infusion]) also counts as a trial of one systemic agent for UC
OR
(b) Member has severe disease indicated by one or more of the following: > 6 stools per day, frequent bloody stools, frequent urgency or incontinence, laboratory abnormalities (i.e., anemia, elevated CRP, low albumin), severe indicators on endoscopy or imaging (i.e., mucosal inflammation, extensive colitis), corticosteroid dependent or refractory to oral corticosteroids
OR
(c) Documentation the member has pouchitis AND has tried therapy with an antibiotic (e.g., metronidazole, ciprofloxacin), probiotic, corticosteroid enema or topical mesalamine
AND
3. Documentation of prescriber baseline disease activity evaluation and goals for treatment to be used to evaluate efficacy of therapy at renewal [DOCUMENTATION REQUIRED]

H. ANKYLOSING SPONDYLITIS
1. Documented diagnosis of ankylosing spondylitis
AND
2. Documentation of treatment failure, serious side effects or clinical contraindication to TWO NSAIDs (e.g., ibuprofen, naproxen, etodolac, meloxicam, indomethacin) for ≥3 consecutive months at maximal recommended or tolerated anti- inflammatory doses
AND
3. FOR MEMBER WITH PROMINENT PERIPHERAL ARTHRITIS: Documentation of treatment failure, serious side effects or clinical contraindication to a trial (≥3 consecutive months) of methotrexate OR sulfasalazine
AND
4. Documentation of prescriber baseline disease activity evaluation and goals for treatment to be used to evaluate efficacy of therapy at renewal [DOCUMENTATION REQUIRED]

I. HIDRADENITIS SUPPURATIVA
1. Documentation of diagnosis of Hidradenitis suppurativa Hurley stage II (moderate recurrent) or stage III (severe diffuse) disease (see Appendix 2)

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AND

2. Documentation indicating the member has been counseled on the use of general supportive measures (e.g., education and support, avoidance of skin trauma, hygiene, dressings, smoking cessation, weight management, diet)
   AND
3. (a) Documentation of treatment failure with or a clinical contraindication to a 3-month trial of the following: an oral antibiotic (e.g., minocycline, doxycycline, clindamycin/rifampin), AND an antiandrogen (e.g., finasteride)
   AND
   (b) Documentation of treatment failure with or a clinical contraindication to intralesional corticosteroids
   AND
4. Documentation of prescriber baseline disease activity evaluation and goals for treatment to be used to evaluate efficacy of therapy at renewal [DOCUMENTATION REQUIRED]

J. NON-INFECTIOUS UVEITIS
1. Documentation of diagnosis of non-infectious intermediate, posterior, or pan-uveitis
   AND
2. Documentation of prescriber baseline disease activity evaluation and goals for treatment to be used to evaluate efficacy of therapy at renewal [DOCUMENTATION REQUIRED]
   AND
3. (a) Documentation of treatment failure, serious side effects or clinical contraindication to ONE of the following: (i) an intravitreal steroid (e.g., triamcinolone, dexamethasone) OR (ii) a systemic corticosteroid (e.g., prednisone, methylprednisolone) OR (iii) an anti-metabolite (e.g., methotrexate, azathioprine, mycophenolate) OR (iv) a calcineurin inhibitor (e.g., cyclosporine, tacrolimus)
   OR
   (b) Documentation of severe uveitis associated with Behcet's syndrome

K. PYODERMA GANGRENOSUM:
1. Documentation of diagnosis of pyoderma gangrenosum
   AND
2. Documentation of prescriber baseline disease activity evaluation and goals for treatment to be used to evaluate efficacy of therapy at renewal [DOCUMENTATION REQUIRED]
   AND
3. Documentation of a trial (≥ 6 weeks) and failure or labeled contraindication to formulary systemic glucocorticoids AND systemic cyclosporine

CONTINUATION OF THERAPY:

A. ALL INDICATIONS:
1. Adherence to therapy at least 85% of the time as verified by the prescriber or member medication fill history OR adherence less than 85% of the time due to the need for surgery or treatment of an infection, causing temporary discontinuation
   AND
2. Prescriber attests to or clinical reviewer has found no evidence of intolerable adverse effects or drug toxicity
   AND
3. Documentation of positive clinical response as demonstrated by low disease activity and/or improvements in the condition's signs and symptoms. [DOCUMENTATION REQUIRED]
   NOTE: Discontinue therapy in adult patients with Ulcerative Colitis without evidence of clinical remission by eight weeks of therapy per FDA label.
   AND
4. Prescriber attests to ongoing monitoring for development of infection (e.g., tuberculosis, Hepatitis B reactivation, etc.) according to the FDA label

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ALL INDICATIONS:
Initial authorization and continuation of therapy: 12 months

PRESCRIBER REQUIREMENTS:
[If prescribed in consultation, consultation notes must be submitted with initial request and reauthorization requests]

Indication	Prescribed by or in consultation with a board-certified
Rheumatoid Arthritis Juvenile Idiopathic Arthritis Ankylosing Spondylitis	Rheumatologist
Psoriatic Arthritis Plaque Psoriasis Pyoderma Gangrenosum	Rheumatologist or Dermatologist
Crohn's Disease Ulcerative Colitis	Gastroenterologist or Colorectal surgeon
Hidradenitis Suppurativa	Dermatologist
Non-infectious Uveitis	Ophthalmologist

AGE RESTRICTIONS:
[Verify product specific labeling]

Indication	Age Restriction
Juvenile Idiopathic Arthritis Non-infectious Uveitis	2 years of age and older
Plaque Psoriasis	4 years of age and older
Ulcerative Colitis	5 years of age and older
Crohn's Disease	6 years of age and older
Hidradenitis Suppurativa	12 years of age and older
Rheumatoid Arthritis Psoriatic Arthritis Ankylosing Spondylitis Pyoderma Gangrenosum	18 years of age and older

QUANTITY:

Medication	Standard Limit	FDA-recommended dosing
10 mg/0.1 mL single-use prefilled syringe	2 syringes per 28 days	**RA/PsA/AS** 40 mg every other week For patients not taking concomitant methotrexate: may increase to 40 mg every week or 80 mg every other week if needed **JIA/Pediatric uveitis (2 years and up)** 10 kg to < 15 kg: 10 mg every other week 15 kg to < 30 kg: 20 mg every other week
10 mg/0.2 mL single-use prefilled syringe	2 syringes per 28 days	
20 mg/0.2 mL single-use prefilled syringe	2 syringes per 28 days	

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20 mg/0.4 mL single-use prefilled syringe	2 syringes per 28 days	≥ 30 kg: 40 mg every other week

40 mg/0.4 mL single-use prefilled syringe/pen	4 syringes/pens per 28 days	**Pediatric CD (6 years and up)** 17 kg to < 40 kg: Loading doses of 80 mg on day 1 and 40 mg (on day 15; Maintenance dose of 20 mg every other week starting on day 29
40 mg/0.8 mL single-use prefilled syringe/pen	4 syringes/pens per 28 days	≥ 40 kg: Loading doses of 160 mg on day 1 (single dose or split over two consecutive days) and 80 mg (on day 15; Maintenance dose of 40 mg every other week starting on day 29 **Pediatric plaque psoriasis (off-label dosing)** 0.8 mg/kg (max dose 40mg) at weeks 0 and 1, then every other week
40 mg/0.8 mL Pediatric Crohn's Disease Starter Pack	6 syringes per 28 days	
80 mg/0.8 mL Pediatric Crohn's Disease Starter Pack	3 syringes per 28 days	**Adult CD and UC** Loading doses: 160 mg on day 1 (given in one day or split over two consecutive days), followed by 80 mg two on day 15 Maintenance dose: 40 mg every other week starting on day 29 **Pediatric UC (5 years and up)** 20 kg to < 40 kg: Loading doses: 80 mg on day 1, 40 mg on days 8 and 15 Maintenance dose: 40 mg every other week or 20 mg every week starting on day 29* ≥ 40 kg: Loading doses: 160 mg on day 1 (single dose or split over two consecutive days), 80mg on days 8 and 15 Maintenance dose: 80 mg every other week or 40 mg every week starting on day 29* *Continue the recommended pediatric dosage in patients who turn 18 years of age and who are well-controlled on their adalimumab regimen.
80 mg/0.8 mL and 40 mg/0.4 mL Pediatric Crohn's Disease Starter Pack	2 syringes per 28 days	
40 mg/0.8 mL pen Crohn's Disease, Ulcerative Colitis, or Hidradenitis Suppurativa Starter Pack	6 pens per 28 days	**Plaque psoriasis/ Adult Uveitis** 80 mg initial dose, followed by 40 mg every other week starting one week after the initial dose **Adolescent hidradenitis suppurativa (12 years and up)** 30 kg to < 60 kg: 80 mg on day 1, 40 mg on day 8 and subsequent doses 40 mg every other week ≥ 60 kg: Follow adult dosing **Adult hidradenitis suppurativa** Loading doses: 160 mg on day 1 (given in one day or split over two consecutive days), followed by 80mg on day 15 Maintenance dose: 40 mg every week or 80 mg every other week starting on day 29
80 mg/0.8 mL pen Crohn's Disease, Ulcerative Colitis, or Hidradenitis Suppurativa Starter Pack	3 pens per 28 days	
40 mg/0.8 mL pen Psoriasis/Uveitis/Adolescent Hidradenitis Suppurativa Starter Pack	4 pens per 28 days	

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80 mg/0.8 mL and 40 mg/0.4 mL Psoriasis/Uveitis/Adolescent Hidradenitis Suppurativa Starter Pack	3 pens per 28 days	**Adult Pyoderma gangrenosum:** SC: 40 mg every week or 80 mg every other week

When requests for off-label dosing, dose escalation, or dose intensification are received, requests will be reviewed for evidence that current or standard dosing is not adequate to produce a therapeutic level of drug (e.g., pharmacokinetic failure), clinical failure or significant loss of response is present, and the requested dosing is established as safe and effective for the condition. There are certain situations where no additional amount of drug is likely to produce or recapture clinical effect because the condition is no longer responsive to the drug (e.g., pharmacodynamic failure) or the drug cannot reach the site of activity at sufficient levels. Review the following items to determine if the requested dosing is medically necessary:

a. *FDA or compendium-supported dosing and therapeutic monitoring recommendations for the drug*
   AND
b. *Member claims/adherence history*
   AND
c. *Clinical documentation of the member's response to current or standard dosing regimens (disease activity indices if commonly used in clinical practice or documentation to approximate them may be necessary to demonstrate the response)*
   AND
d. *In conjunction with documented clinical failure or loss of response or wearing off of effect, test results that demonstrate failure of current or standard dosing to reach established treatment thresholds (e.g., established therapeutic monitoring recommendations)*
   AND
e. *If applicable, documentation showing the member does not have conditions which make achieving a therapeutic level of drug unlikely even with dose intensification (eg, dose intensification may be futile due to the presence of anti-drug antibodies, protein losing enteropathy, nephrotic syndrome, severe drug excretion or malabsorption issues, etc.)*
   AND
f. *In certain situations, documentation, or peer-to-peer determination that re-induction cannot be tried to recapture response as an alternative to long term dose escalation or intensification*

PLACE OF ADMINISTRATION:
The recommendation is that injectable medications in this policy will be for pharmacy benefit coverage and patient self-administered.

DRUG INFORMATION

ROUTE OF ADMINISTRATION:
Subcutaneous

DRUG CLASS:
Anti-TNF-Alpha-Monoclonal Antibodies

FDA-APPROVED USES:
Indicated for:
- Reducing signs and symptoms, inducing major clinical response, inhibiting the progression of structural damage, and improving physical function in adult patients with moderately to severely active Rheumatoid Arthritis (RA).
- Reducing signs and symptoms of moderately to severely active polyarticular Juvenile Idiopathic Arthritis (JIA) in patients 2 years of age and older.
- Reducing signs and symptoms, inhibiting the progression of structural damage, and improving physical function in adult patients with active Psoriatic Arthritis (PsA).
- Reducing signs and symptoms in adult patients with active Ankylosing Spondylitis (AS).
- Treatment of moderately to severely active Crohn's Disease (CD) in adults and pediatric patients 6 years of

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age and older.
- Treatment of moderately to severely active Ulcerative Colitis (UC) in adults and pediatric patients 5 years of age and older. *Limitations of Use: Effectiveness has not been established in patients who have lost response to or were intolerant to TNF blockers.*
- Treatment of adult patients with moderate to severe chronic Plaque Psoriasis (Ps) who are candidates for systemic therapy or phototherapy, and when other systemic therapies are medically less appropriate.
- Treatment of moderate to severe Hidradenitis Suppurativa (HS) in patients 12 years of age and older.
- Treatment of non-infectious intermediate, posterior, and panuveitis in adults and pediatric patients 2 years of age and older

COMPENDIAL APPROVED OFF-LABELED USES:
Pyoderma gangrenosum, Systemic juvenile idiopathic arthritis, Moderate to severe psoriasis in children and adolescents

APPENDIX

APPENDIX 1:
OBJECTIVE MEASURES FOR RA:
[Clinical Disease Activity Index (CDAI), Disease Activity Score with 28-joint counts (erythrocyte sedimentation rate or C-reactive protein), Member Activity Scale (PAS or PAS-II), Routine Assessment of Member Index Data with 3 measures, Simplified Disease Activity Index (SDAI)]

OBJECTIVE MEASURES FOR PJIA:
Global Arthritis Score (GAS), Clinical Disease Activity Index (CDAI), Disease Activity Score (DAS), Disease Activity Score based on 28-joint evaluation (DAS28), Simple Disease Activity Index (SDAI), Health Assessment Questionnaire disability index (HAQ-DI), Visual Analogue Scale (VAS), Likert scales of global response or pain by the member or global response by the physician, Joint tenderness and/or swelling counts, Laboratory data

APPENDIX 2:
Hurley Staging (Alikhan, 2019):
Hurley stage I is characterized by recurrent nodules and abscesses with minimal scarring.
Hurley stage II is characterized by nodules and abscesses and 1 or a limited number of sinuses/tunnels, and/or scarring within a body region.
Hurley stage III is characterized by multiple or extensive nodules and abscesses, sinuses/tunnels, and/or scarring.

APPENDIX 3:
A biosimilar is highly similar version of a brand name biological drug that meets strict controls for structural, pharmaceutical, and clinical consistency. A biosimilar manufacturer must demonstrate that there are no meaningful clinical differences (i.e., safety and efficacy) between the biosimilar and the reference product. Clinical performance is demonstrated through human pharmacokinetic (exposure) and pharmacodynamic (response) studies, an assessment of clinical immunogenicity, and, if needed, additional clinical studies.[1] As costs for biological specialty drugs continue to rise, the growing biosimilar market will benefit providers and patients by broadening biological treatment options and expanding access to these medications at lower costs. Molina Healthcare, Inc. continues to be committed to continually reevaluating preferred strategies and applying innovative cost-controls to ensure patients receive safe, effective, and quality healthcare. This commitment includes potentially creating a preference for biosimilars when value can be added without compromising patient satisfaction and safety.

1. Food and Drug Administration. Biosimilar and Interchangeable Products. Retrieved from https://www.fda.gov/drugs/biosimilars/biosimilar-and-interchangeable-products. Accessed October 8, 2019

BACKGROUND AND OTHER CONSIDERATIONS

BACKGROUND:
Humira (adalimumab) is a monoclonal antibody that binds to tumor necrosis factor alpha. Elevated levels of TNF contribute to pain and joint destruction in immune-mediated arthritis. Humira binds to TNF alpha

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receptor sites to interfere with the inflammation process. Humira is indicated for reducing signs and symptoms, inducing major clinical response, inhibiting the progression of structural damage, and improving physical function in adult patients with moderately to severely active rheumatoid arthritis. It is indicated for reducing signs and symptoms of moderately to severely active polyarticular juvenile idiopathic arthritis in pediatric patients 2 years of age and older. Humira is also indicated for reducing signs and symptoms, inhibiting the progression of structural damage, and improving physical function in adult patients with active psoriatic arthritis. It is indicated for reducing signs and symptoms in adult patients with active ankylosing spondylitis. Humira is indicated for reducing signs and symptoms and inducing and maintaining clinical remission in adult patients with moderately to severely active Crohn's disease who have had an inadequate response to conventional therapy and reducing signs and symptoms and inducing clinical remission in these patients if they have also lost response to or are intolerant to infliximab. Humira is indicated for reducing signs and symptoms and inducing and maintaining clinical remission in pediatric patients 6 years of age and older with moderately to severely active Crohn's disease who have had an inadequate response to corticosteroids or immunomodulators such as azathioprine, 6-mercaptopurine, or methotrexate. Humira is also indicated for inducing and sustaining clinical remission in adult patients with moderately to severely active ulcerative colitis who have had an inadequate response to immunosuppressants such as corticosteroids, azathioprine or 6-mercaptopurine (6-MP). Humira is indicated for the treatment of adult patients with moderate to severe chronic plaque psoriasis who are candidates for systemic therapy or phototherapy, and when other systemic therapies are medically less appropriate. Humira is indicated for the treatment of moderate to severe hidradenitis suppurativa. Finally, Humira is indicated for the treatment of noninfectious intermediate, posterior and panuveitis in adult patients.

Pyoderma Gangrenosum is an inflammatory skin disorder. The exact cause is unknown; however, it is thought to be autoimmune. Approach of treatment depends on severity. Initial treatment for mild cases are typically a high potency or super potent topical steroid or topical tacrolimus. Patients who fail this or have more extensive disease should be treated with systemic treatment such as systemic glucocorticoids, cyclosporine. Humira (adalimumab) has also been noted to be beneficial in members with pyoderma gangrenosum. (Schadt, C)

AGA Guidelines Moderate to Severe Ulcerative Colitis

Recommendations from the recent 2020 guideline update include:

- In adult outpatients with moderate to severe UC who are naïve to biologic agents, the AGA suggests using infliximab or vedolizumab rather than adalimumab, for induction of remission.
- Updated FDA recommendations (July 26, 2019) on indications for use of tofacitinib in UC recommends its use only after failure of or intolerance to TNF-a antagonists.
- In adult outpatients with moderate to severe UC who have previously been exposed to infliximab, particularly those with primary nonresponse, the AGA suggests using ustekinumab or tofacitinib rather than vedolizumab or adalimumab for induction of remission.
- In adult outpatients with moderate to severe UC, the AGA suggests against using methotrexate monotherapy for induction or maintenance of remission
- In adult outpatients with active moderate to severe UC, the AGA suggests using biologic monotherapy (TNF-a antagonists, vedolizumab, or ustekinumab) or tofacitinib rather than thiopurine monotherapy for induction of remission.
- In adult outpatients with moderate to severe UC, the AGA suggests combining TNF-a antagonists, vedolizumab or ustekinumab with thiopurines or methotrexate rather than biologic monotherapy.
- In adult outpatients with moderate to severe UC who have achieved remission with biologic agents and/or immunomodulators or tofacitinib, the AGA suggests against continuing 5-ASA for induction and maintenance of remission.

VARSITY Trial: Ulcerative Colitis in Adults

Entyvio and Humira were compared in adults with moderately to severely active UC in a Phase 3b double-blind, double-dummy, randomized trial. Previous exposure to a tumor necrosis factor inhibitor other than adalimumab was allowed in up to 25% of patients. At week 52, clinical remission was observed in a higher percentage of patients in the Entyvio group than in the Humira group (31.3% vs. 22.5%; difference, 8.8

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percentage points; 95% confidence interval [CI], 2.5 to 15.0; P=0.006), as was endoscopic improvement (39.7% vs. 27.7%; difference, 11.9 percentage points; 95% CI, 5.3 to 18.5; P<0.001). This data and the current AGA guidelines that suggest using infliximab or Entyvio rather than adalimumab, for induction of remission, show Entyvio has emerged as a first-line agent for the management of moderately to severely active UC.

CONTRAINDICATIONS/EXCLUSIONS/DISCONTINUATION:
All other uses of Humira (adalimumab) are considered experimental/investigational and therefore, will follow Molina's Off- Label policy. Contraindications to Humira (adalimumab) include: No labeled contraindications. Warnings/precautions of Humira (adalimumab) include: Serious infections: Do not start Humira during an active infection. If an infection develops, monitor carefully, and stop Humira if infection becomes serious. Invasive fungal infections: For patients who develop a systemic illness on Humira, consider empiric antifungal therapy for those who reside or travel to regions where mycoses are endemic.

Malignancies: Incidence of malignancies was greater in Humira-treated patients than in controls. Anaphylaxis or serious hypersensitivity reactions may occur. Hepatitis B virus reactivation: Monitor HBV carriers during and several months after therapy. If reactivation occurs, stop Humira and begin antiviral therapy. Demyelinating disease: Exacerbation or new onset, may occur.

Cytopenias, pancytopenia: Advise patients to seek immediate medical attention if symptoms develop and consider stopping Humira. Heart failure: Worsening or new onset, may occur. Lupus-like syndrome: Stop Humira if syndrome develops.

OTHER SPECIAL CONSIDERATIONS:
Humira has a Black box warning for serious infections and malignancy. Increased risk of serious infections leading to hospitalization or death, including tuberculosis (TB), bacterial sepsis, invasive fungal infections (such as histoplasmosis), and infections due to other opportunistic pathogens. Discontinue Humira if a patient develops a serious infection or sepsis during treatment. Perform test for latent TB; if positive, start treatment for TB prior to starting Humira. Monitor all patients for active TB during treatment, even if initial latent TB test is negative. Lymphoma and other malignancies, some fatal, have been reported in children and adolescent patients treated with TNF blockers including Humira. Post-marketing cases of hepatosplenic T-cell lymphoma (HSTCL), a rare type of T-cell lymphoma, have occurred in adolescent and young adults with inflammatory bowel disease treated with TNF blockers including Humira.

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HCPCS CODE	DESCRIPTION
NA	

AVAILABLE DOSAGE FORMS:

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Abrilada (1 Pen) AJKT 40MG/0.8ML	Hulio (2 Pen) AJKT 40MG/0.8ML
Abrilada (2 Pen) AJKT 40MG/0.8ML	Hulio (2 Syringe) PSKT 20MG/0.4ML
Abrilada (2 Syringe) PSKT 20MG/0.4ML	Hulio (2 Syringe) PSKT 40MG/0.8ML
Abrilada (2 Syringe) PSKT 40MG/0.8ML	Humira (1 Pen) AJKT 80MG/0.8ML
Adalimumab-aacf (2 Pen) AJKT 40MG/0.8ML	Humira (2 Pen) AJKT 40MG/0.4ML
Adalimumab-aacf (2 Syringe) PSKT 40MG/0.8ML	Humira (2 Pen) AJKT 40MG/0.8ML
Adalimumab-aacf(CD/UC/HS Strt) AJKT 40MG/0.8ML	Humira (2 Pen) AJKT 80MG/0.8ML
Adalimumab-aacf(Ps/UV Starter) AJKT 40MG/0.8ML	Humira (2 Syringe) PSKT 10MG/0.1ML
Adalimumab-aaty (1 Pen) AJKT 40MG/0.4ML	Humira (2 Syringe) PSKT 20MG/0.2ML
Adalimumab-aaty (1 Pen) AJKT 80MG/0.8ML	Humira (2 Syringe) PSKT 40MG/0.4ML
Adalimumab-aaty (2 Pen) AJKT 40MG/0.4ML	Humira (2 Syringe) PSKT 40MG/0.8ML
Adalimumab-aaty (2 Syringe) PSKT 20MG/0.2ML	Humira-CD/UC/HS Starter AJKT 40MG/0.8ML
Adalimumab-aaty (2 Syringe) PSKT 40MG/0.4ML	Humira-CD/UC/HS Starter AJKT 80MG/0.8ML
Adalimumab-aaty CD/UC/HS Start AJKT 80MG/0.8ML	Humira-Ped<40kg Crohns Starter PSKT 80 MG/0.8ML &40MG/0.4ML
Adalimumab-adaz SOAJ 40MG/0.4ML	Humira-Ped>/=40kg Crohns Start PSKT 80MG/0.8ML
Adalimumab-adaz SOAJ 80MG/0.8ML	Humira-Ped>/=40kg UC Starter AJKT 80MG/0.8ML
Adalimumab-adaz SOSY 10MG/0.1ML	Humira-Ps/UV/Adol HS Starter AJKT 40MG/0.8ML
Adalimumab-adaz SOSY 20MG/0.2ML	Humira-Psoriasis/Uveit Starter AJKT 80 MG/0.8ML &40MG/0.4ML
Adalimumab-adaz SOSY 40MG/0.4ML	Hyrimoz SOAJ 40MG/0.4ML
Adalimumab-adbm (2 Pen) AJKT 40MG/0.4ML	Hyrimoz SOAJ 40MG/0.8ML
Adalimumab-adbm (2 Pen) AJKT 40MG/0.8ML	Hyrimoz SOAJ 80MG/0.8ML
Adalimumab-adbm (2 Syringe) PSKT 10MG/0.2ML	Hyrimoz SOSY 10MG/0.1ML
Adalimumab-adbm (2 Syringe) PSKT 20MG/0.4ML	Hyrimoz SOSY 20MG/0.2ML
Adalimumab-adbm (2 Syringe) PSKT 40MG/0.4ML	Hyrimoz SOSY 40MG/0.4ML
Adalimumab-adbm (2 Syringe) PSKT 40MG/0.8ML	Hyrimoz SOSY 40MG/0.8ML
Adalimumab-adbm(CD/UC/HS Strt) AJKT 40MG/0.4ML	Hyrimoz-Crohns/UC Starter SOAJ 80MG/0.8ML
Adalimumab-adbm(CD/UC/HS Strt) AJKT 40MG/0.8ML	Hyrimoz-Ped<40kg Crohn Starter SOSY 80 MG/0.8ML &40MG/0.4ML
Adalimumab-adbm(Ps/UV Starter) AJKT 40MG/0.4ML	Hyrimoz-Ped>/=40kg Crohn Start SOSY 80MG/0.8ML
Adalimumab-adbm(Ps/UV Starter) AJKT 40MG/0.8ML	Hyrimoz-Plaq Psor/Uveit Start SOAJ 80 MG/0.8ML &40MG/0.4ML
Adalimumab-bwwd SOAJ 40MG/0.4ML	Hyrimoz-Plaque Psoriasis Start SOAJ 80 MG/0.8ML &40MG/0.4ML
Adalimumab-bwwd SOSY 40MG/0.4ML	Idacio (2 Pen) AJKT 40MG/0.8ML
Adalimumab-fkjp (2 Pen) AJKT 40MG/0.8ML	Idacio (2 Syringe) PSKT 40MG/0.8ML
Adalimumab-fkjp (2 Syringe) PSKT 20MG/0.4ML	Idacio-Crohns/UC Starter AJKT 40MG/0.8ML
Adalimumab-fkjp (2 Syringe) PSKT 40MG/0.8ML	Idacio-Psoriasis Starter AJKT 40MG/0.8ML
Adalimumab-ryvk (1 Pen) AJKT 80MG/0.8ML	Simlandi (1 Pen) AJKT 40MG/0.4ML
Adalimumab-ryvk (2 Pen) AJKT 40MG/0.4ML	Simlandi (1 Pen) AJKT 80MG/0.8ML
Adalimumab-ryvk (2 Syringe) PSKT 40MG/0.4ML	Simlandi (1 Syringe) PSKT 80MG/0.8ML
Amjevita SOAJ 40MG/0.4ML	Simlandi (2 Pen) AJKT 40MG/0.4ML
Amjevita SOAJ 40MG/0.8ML	Simlandi (2 Syringe) PSKT 20MG/0.2ML
Amjevita SOAJ 80MG/0.8ML	Simlandi (2 Syringe) PSKT 40MG/0.4ML
Amjevita SOSY 40MG/0.4ML	Yuflyma (1 Pen) AJKT 40MG/0.4ML
Amjevita SOSY 40MG/0.8ML	Yuflyma (1 Pen) AJKT 80MG/0.8ML
Amjevita-Ped 10kg to <15kg SOSY 10MG/0.2ML	Yuflyma (2 Pen) AJKT 40MG/0.4ML
Amjevita-Ped 15kg to <30kg SOSY 20MG/0.2ML	Yuflyma (2 Syringe) PSKT 20MG/0.2ML
Amjevita-Ped 15kg to <30kg SOSY 20MG/0.4ML	Yuflyma (2 Syringe) PSKT 40MG/0.4ML
Cyltezo (2 Pen) AJKT 40MG/0.4ML	Yuflyma-CD/UC/HS Starter AJKT 80MG/0.8ML
Cyltezo (2 Pen) AJKT 40MG/0.8ML	Yusimry SOAJ 40MG/0.8ML
Cyltezo (2 Syringe) PSKT 10MG/0.2ML	
Cyltezo (2 Syringe) PSKT 20MG/0.4ML	
Cyltezo (2 Syringe) PSKT 40MG/0.4ML	
Cyltezo (2 Syringe) PSKT 40MG/0.8ML	
Cyltezo-CD/UC/HS Starter AJKT 40MG/0.4ML	
Cyltezo-CD/UC/HS Starter AJKT 40MG/0.8ML	
Cyltezo-Psoriasis/UV Starter AJKT 40MG/0.4ML	
Cyltezo-Psoriasis/UV Starter AJKT 40MG/0.8ML	
Hadlima PushTouch SOAJ 40MG/0.4ML	
Hadlima PushTouch SOAJ 40MG/0.8ML	
Hadlima SOSY 40MG/0.4ML	
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SUMMARY OF REVIEW/REVISIONS	DATE
Off-Cycle Review – Notable Revisions: Required Medical Information Continuation of Therapy Appendix References	10/2026
Annual Review – Notable Revisions: Products Affected FDA-Approved Uses Available Dosage Forms References	08/2026
NEW CRITERIA CREATION	07/2025

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